Baylor University Medical Center Proceedings
The peer-reviewed journal of Baylor Scott & White Health

ISSN: 0899-8280 (Print) 1525-3252 (Online) Journal homepage: http://www.tandfonline.com/loi/ubmc20

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To cite this article: Pamela G. Sanford & Phyllis Hart Tipton (2016) Is Nursing Preceptor Behavior Changed by Attending a Preceptor Class?, Baylor University Medical Center Proceedings, 29:3, 277-279, DOI: 10.1080/08998280.2016.11929434

To link to this article: https://doi.org/10.1080/08998280.2016.11929434

Published online: 11 Dec 2017.

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Is nursing preceptor behavior changed by attending a preceptor class?

Pamela G. Sanford, MSN, RN-BC, CNS, and Phyllis Hart Tipton, PhD, RN

Nursing preceptors initiate new staff to the professional environment. To be successful, preceptors must be willing and knowledgeable in their role, both clinically and as an instructor. This study evaluated the effects of a 4-hour preceptor class to change preceptor behavior. Twenty-seven class participants commented on the class and their goals, and 18 completed a follow-up survey 2 months later. Among those 18, 90% had achieved at least one of their goals. One barrier to achieving planned changes was having the opportunity to precept over the time period. Participants indicated that improved listening skills, application of content, and team building all supported their improvement as a preceptor.

Preceptors and preceptorships have been a part of nursing for many years. Preceptors teach students across a variety of acute, community, and continuing care practices (1). They provide a safety net of experienced staff to answer questions and provide insight (2). While there are multiple considerations for providing a preceptor experience, one study identified three themes associated with good preceptorships: a caring relationship, mutual respect, and a deep sense of responsibility (3).

Dusaj described the importance of the preceptor-preceptee relationship in ensuring a successful orientation (4). Specific actions of the preceptor include providing a tool box of resources and actions that prepare the new staff member to be successful. Preceptors also boost morale and confidence while making the new nurse feel a part of the team. Many authors depict preceptors as having the experience to guide new nurses as they assimilate into the new work environment. Galsper stressed the importance of the preceptor having experience in the areas needed by the preceptee (5). In interviews with preceptors, Hilli et al concluded that a good preceptor has years of work experience, allowing a sense of security in the role of nurse (3). This is in line with the study of Chen, Li-Ling, and Suh-Ing, who identified the best preceptors as older, senior nurses who choose to precept (6). The preceptor-preceptee relationship not only guides and inspires new staff members, but introduces preceptors to ideas they may not have considered (7). Preceptors must be active teachers willing to share ideas and help new nurses search out answers, as well as excellent role models (8, 9). Additionally, they must be able to manage poor performance and develop action plans when needed.

Preceptors face a broad set of teaching-learning diversity issues related to their role as clinical teachers. Being unaware of these issues may contribute to miscommunication and conflict. However, preceptors often have limited information on this diversity (8). The implication for nursing education is to prepare the preceptor for the role and thereby increase satisfaction and improve the preceptee's experience. In a study by Kalischuck et al, preceptors expressed the need for a lighter workload, further educational preparation, and more time assessing and assisting students (1). They also felt a need for increased support of stakeholders. Increased support encourages preceptor retention and influences future nurses. The unprepared preceptor is not confident in the role (10). This lack of confidence could result in a poor transition of the preceptee to staff member, possibly increasing staff turnover and losing the new nurse to the profession. This retrospective study identified potential changes to the practice of nurse preceptors due to education and identified situations that facilitate or hinder such practice change.

METHODOLOGY

To facilitate preceptor development, three preceptor courses were provided in 2013. Each 4-hour class presented by a clinic educator covered the role of the preceptor, learning styles, growth and development of new staff, competency assessment, and critical thinking. Class participants were asked to identify three practice improvements based on course content. A total of 27 attendees identified anticipated changes in behavior due to participation in the preceptor class. Two months after the class, the students were asked to return the outcomes tool identifying which changes had occurred and to relate any barriers to making the changes. Eighteen tools were returned, for a 66% return rate: the January class had 15 attendees, and 13 responded at 2 months; the April class had 6 attendees, and 3 responded at 2 months; and the September class had 6 attendees, and 2 responded at 2 months.

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For the purposes of this paper, *preceptor* is defined as a nursing staff member—registered nurse, licensed vocational nurse, or certified medical assistant—who has attended a preceptor class and is charged with the orientation of a new staff member. The preceptor must have a job role equal to or higher than that of the preceptee. A *preceptorship* is defined as a relationship between an experienced nursing staff member and a newly hired staff member; the length of this relationship depends on the orientation period of the specific nursing unit or clinic.

RESULTS

Upon receiving institutional review board approval, the data were deidentified. The 27 participants were classified only as registered nurses (n = 8; 30%), licensed vocational nurses (n = 13; 48%), certified medical assistants (n = 3; 12%), and nonnursing staff (n = 3; 12%). The participants described long-term goals on the evaluation tool, listing the improvements or changes they planned to make as preceptors based on class learning. Narrative responses on the outcome measurement tool were categorized into three broad themes: 1) preceptor/teaching; 2) orientee/learning; and 3) role of the preceptor. Multiple goals identified by individual participants fell into more than one theme. Several participants commented on the learning from the class: "ensure my body language represents what I am really saying," "teach new staff more about critical thinking and interpersonal skills because it isn't all about technical skills," and "appreciation that the preceptor role extends past orientation."

Eighteen participants (66%) completed the follow-up survey 2 months after the class. Of these, 16 (90%) achieved one or more of their goals. Of these 16, 3 (20%) achieved one goal, 5 (30%) achieved two goals, and 8 (50%) achieved three goals.

The respondents were asked to identify barriers to achieving their planned changes. One barrier was unexpected time away from the clinic so participants were not able to precept during the 2 months. Several stated they had not had the opportunity to practice the skills since no new nurses were hired. One comment was: "I have become generationally sensitive to my peers. The class was helpful in understanding my coworkers and their work ethic. I have not had to precept anyone thus far but will soon."

When describing supportive factors to utilizing the content, three categories were identified: improved listening skills (n = 5), application of content (n = 8, including the ability to build on the preceptee's strengths, generational sensitivity, and the creation of a resource manual), and team building (n = 6, including understanding coworkers and increased involvement). Examples of comments describing how the content was utilized in the work setting include:

- "After attending this class and learning about the different age groups/needs, training new employees has become easier and feel like I am able to reach them on their level due to the breakdown in age groups."
- "I have been able to appreciate everyone's role in the clinical setting. This in turn has helped me as a nurse contribute my strengths and abilities in a stronger way. Having recently had the opportunity to be 'the preceptor' to a young nurse, I was able to direct and be the person outlined in the class with more confidence and assurance."
- "What facilitated the projected changes is the fact that I can be very open with coworkers and we work as a team. They also know they can be very open with me about any concerns."

DISCUSSION

The results of this study suggest that the class was of value in enhancing the preceptor role by developing skills in the participants. Improvement of teaching was mentioned 27 times; this could indicate that basic nursing education does not ensure that staff nurses are ready to precept, and they need more preparation for this responsibility. The class also helped the preceptor realize that the learning needs of the preceptee may go beyond the technical aspects of nursing. Johnston and Mohide and Fowler also identified that preceptors must actively teach and support the new nurse in identifying their own learning needs (8, 9).

Entry-level nursing education is grounded in the nursing process (i.e., assess, plan, implement, and evaluate). The education process is very similar, with the focus moved to staff rather than patients. While patient education and patient teaching are often a component of clinical requirements in school, teaching/precepting peers is not routinely addressed, even at the bachelor's degree level. Therefore, staff nurses need to have preceptor training before they precept. Another finding from our study was recognizing that the role of the preceptor included both leadership and socialization of the new staff nurse. Dusaj and Hilli et al described socialization of the new hire as an important function for the preceptor (3, 4).

Dusaj described developing a tool box of resources for the preceptor (4). Two participants from this study chose to develop a resource manual for their unit during the 2 months between the surveys when they were unable to put their learning into practice. This suggests that they recognized the ongoing process of precepting and the value of having resources to support staff new to the role.

The experienced nurse is often from a different generation than the new staff nurse/student. Therefore, there may be significant differences in the learning styles of the two groups. Preceptors must be able to recognize the strengths and weaknesses of each preceptee to ensure that educational needs are met. This may be quite different from patient teaching because often the patients are of the older generation, possibly similar to the experienced nurse, therefore having complementary learning styles. These generational differences work backwards if the younger nurse is precepting someone from an older generation. Chen's observation that preceptors should have experience in the clinical area supports the thought that the preceptor and preceptee may be of different generations, with different priorities and values (6). Our study results suggest that the course participants recognized the impact of generational differences in the nursing workforce.

Acknowledgments

The authors thank Chandler Carroll for her assistance with manuscript preparation.

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